جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سهير سلامة عبد المجيد احمد
المحافظة : الجيزه **الحي / المدينة :** كرداسه **العنوان :** الجيزة
رقم البطاقة : 27902112102367 **رقم التليفون :** 01129387118 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 11-02-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سهير سلامة عبد المجيد احمد
National ID: 27902112102367

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

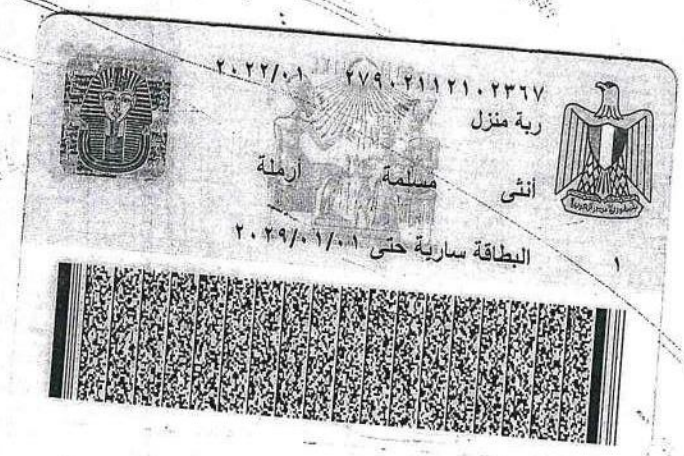
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

7007

Patient's Name:	**soher sa aged**
Date:	**2026-04-11**
Referred by:	Prof.Dr. **ct cap iv**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED;

compared to previous study dated 2044/2025, the current study reveled

I- CHEST:

- Still seen right lower lobe pulmonary nodule is seen measuring 7 mm.
- Resolution of the right minimal pleural effusion
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Mesenteric panniculitis with multiple mesenteric lymph nodes largest measuring 17 x 8 mm.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Few subcentimeteric bilateral external iliac lymph nodes largest measuring 22 x 8 mm at the right external iliac region.
- No ascites.
- Still seen widespread multiple mixed lytic& sclerotic lesions in the axial& appendicular skeleton

Opinion:

❖ **Stationary course**

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

Name: سهير سلامه عبدالمجيد	
MR. : 27	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

patient on arimedex zoladex + zometa for zometa 4mg

prem bilateral breast cancer ct4 n1 m1 met to bone by cts initially and pleura peritoneum diagnosed in 2024 idc 2 er8/8 pr8/8 her2-ve both sidex rec zoladex arimidex from 11/2024 initial cts pleural eff ascites subcentim pul nodules multilocular ovarian cyst lytic osseous lesions bs free bs 12/2025 multiple osseous lesions cts on 10/2025 rd for new cts chest xray cc dyspnea for zoladex arimidex for 1 month

CT CAP 11\4\2026 >> SD ARAMIDEX + SUPP >> 3M ZOMETA TODAY AFTER LABS >> PHARMACY ZOLADEX TO BE TAKEN ON 13\6\2026

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Sohair Salama Abd Al Maguid	Date : 05/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 46 Year(s)
Medical ID: 3375/2024	Pat. ID: 80337

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

COMPARISON : Compared to previous study dated 20/04/2025

FINDINGS :

1. **CHEST:**

- Regressive course of the Bilateral breast heterogenous parenchymal distortion with mild skin thickening .
- Few subcentimetric bilateral axillary enlarged lymph nodes.
- Complete resolution of right pleural effusion.
- Still seen right lower lobe posterior segment pulmonary nodule measuring about 6 mm.
- No pathological hilar or mediastinal lymph nodes.
- Average cardiac size. No pericardial effusion detected.

2. **Abdomen and pelvis:**

- The liver is mild **enlarged** with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Total resolution of Ascites with still smudging of peritoneal fat and nodular small densities
- Still seen mild enlarged both adnexa.
- Still seen mild circumferential gastric wall thickening , no intraluminal masses .

3. **The scanned skeleton:**

- The scanned skeleton shows stable widespread multiple mixed lytic or sclerotic lesions in axial and appendicular skeleton

OPINION:

- **As compared to previous study , the current study shows overall regressive course.**

DR/Reeham.Khader

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Soheir Salama AbdulMajid Ahmed
Branch : Dokki

Date : 01-12-2025 12:29
Medical No. : 263007

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History

- A 46-year-old lady, with history of metastatic breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Multiple foci of mildly increased tracer uptake at multi-levels of the spine, sternum and pelvic bones.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **Mild residual tracer active osseous deposits as described, for follow up**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[DOCTYPE:BONE_SCAN]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : سهير سلامة عبدالمجيد احمد
Sex: Female
3- Medical number: 83235124
Old Med : 3375-2024
2-ID numbers:
Patient Age : 46 Y

Family history	Family History	Phone	01159679310	BMI	BMI	REASON OF MDT	
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :	Mets Workup :
	cap: rt pulmonary nodules+rt pleural effusion mild ascites+peritoneal nodules bilateral adenxal masses bone mets

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	8/8	1		
BX COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	KI

T	N	M
		1 BONE,LUNG

Sonomammography

PATIENT HISTORY

De-novo bilateral T4b breast cancer, IDC, BOTH sides strong HR+ve and HER2 -ve
CT CAP: rt pulmonary nodules+rt pleural effusion ,mild ascites+peritoneal nodules
bilateral BULKY adnexa ,bone mets

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

ZO' ADEX-AI-ZOMETA
Trans vaginal ultrasound to asses adenxae
bone scan and consider palliative RTH according
pain management ,, follow up tumor markers every 3 months

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/شريف حسانين د/يحيى اسماعيل	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / سهير سلامه عبد المجيد احمد	Request No.	: 13003844
Patient No.	: 27902112102367	Request Date	: 22/08/2024
Path No.	: 3044/24	Verification Date	: 26/09/2024
Sex	: Female	Financial Category	: [illegible] - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: marwa mohamed [illegible]

Pathology Report

Clinical Data: Bilateral breast masses
Nature of Specimen: Tru-cut biopsies, collected at 22/8/2024, 10:30 am

Gross Examination:
1- Right breast: Multiple grayish white tissue cores up to 0.7 cm, totally submitted.
2- Left breast: Multiple grayish white tissue cores up to 0.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination of both biopsies:
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2=7) Grade II
* DCIS: No
* IPT (Positive control showed appropriate reactivity): (On Right Breast)
- E-Cadherin: Positive.
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- Her2: No membrane reaction in tumor cells (score 0)
- Ki67: 20% nuclear reaction of tumor cells
* IPT (Positive control showed appropriate reactivity): (On Left Breast)
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+)
- Ki67: 30% nuclear reaction of tumor cells

Diagnosis:
Bilateral breast masses, Tru-cut biopsies:

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	:الاسكندرية / مهير سلامه عبد المجيد احمد	Request No.	: 13003844
Patient No.	:279021121023 67	Request Date	: 22/08/2024
Path No.	: 3044/24	Verification Date	:26/09/2024
Sex	: Female	Financial Category	: اوراده نور السلام - بطاقة الخزانة
Age	: 45 Y	Department	: Out Patient
Refarreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Invasive duct carcinoma, Grade II
IPT (On Right Breast):
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 20% of tumor cells.
IF-T (On Left Breast).
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Score 1+.
- Ki67: Positive in 20% of tumor cells.

Signature
Dr. Rana Fathy
Dr. Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Sohair Salama Abd Al Maguid	Date : 01/10/2024
Exam : C.T CHEST	Age : 45 Year(s)
Medical ID: 3375/2024	Pat. ID: 80337

Non CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. ***Chest:***

- Bilateral breast heterogenous parenchymal distortion with diffuse skin thickening .
- Right axillary level I ,II pathologically enlarged lymph nodes the largest measures 3.2x1.2 cm.
- Left axillary level I ,II & III pathologically enlarged lymph nodes the largest measures 2.3x1.3 cm.
- Bilateral prominent subcentimeteric posterior trainagle lymph nodes
- Moderate right pleural effusion.
- Right lower lobe posterior segment tiny pulmonary nodule measuring about 6 mm.
- Right middle lobe consolidative /collapse patchy area with inter
- No pathological hilar or mediastinal lymph nodes.
- Average cardiac size. No pericardial effusion detected.

2. ***Abdomen and pelvis:***

- The liver is **enlarged** with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- **Mild Ascites with smudging of peritoneal fat and nodular densities , the largest measured 1.6x1 cm seen left hypchondrial region**
- **Bilateral both adnexa measuring 3.2x 5.8 cm on left side & 4x4.2 cm on right side**
- **Mild circumferential gastric wall thickening**

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Sohair Salama Abd Al Maguid	Date : 01/10/2024
Exam : C.T CHEST	Age : 45 Year(s)
Medical ID: 3375/2024	Pat. ID: 80337

3. ***The scanned skeleton:***

- **The scanned skeleton shows multiple mixed lytic or sclerotic lesions in axial and appendicular skeleton**

OPINION:

- ***Bilateral breast malignant process with pathological bilateral axillary lymph nodes as described.***
- ***Right pulmonary nodule with moderate right pleural effusion.***
- ***Mild ascites with sumding of fat planes and peritoneal nodularity as described .***
- ***Bilateral bulky adnexa for further US /MRI assessment.***
- **The scanned skeleton shows multiple mixed lytic or sclerotic lesions, likely metastatic .**

Dr. Mai Salem

Signature Prof. Menna Tallah Hanafy
15/10/2024 12:37 م

CT Scan
[[DOCTYPE:CT_SCAN]]

?????, ???? **Patient ID: 10110224012914 DOB: 11-Feb-1979 Date: 11-Nov-2024**
Patient Lastname: [redacted]
Patient Firstname: [redacted]
HIS Patient ID: 10110224012914
RIS Patient ID: 361786
Date of Birth: 11/02/1979

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========
99mTc MDP Bone Scan Report (11/11/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits seen all over the surveyed skeleton.
" Degenerative changes over the dorso-lumbar vertebrae.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion: -
" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Alaa Elsaeed
Omnia Talaat, MD

========== REPORT TEXT ENDE ==========

Bone Scan
[[DOCTYPE:BONE_SCAN]]